CAP: advanced salivary gland cancer

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Advanced salivary gland cancer · NCCP 00615 v2 · Document revised 14 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Advanced salivary gland cancer, after specialist review of histology, treatment choices and goals.

My overall treatment

Day 1 every 21 days; up to 6 cycles

21 day view

Treatment timing in this window

CAP infusion and fluids — Day 1

Treatment

Doxorubicin 50 mg/m², cisplatin 50 mg/m² and cyclophosphamide 500 mg/m² on Day 1, repeated every 21 days for up to 6 cycles, stopping earlier for progression or unacceptable side effects.

Review

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

A typical 21-day cycle

Day 1	Day 2–21
three medicines and fluids	Between visits

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline preparation** NCCP	Full blood count, kidney and liver tests and electrocardiogram (ECG). Heart-pumping assessment with an echo or multigated acquisition (heart pumping) scan (MUGA) scan if over 65 or clinically indicated; doxorubicin requires left ventricular ejection fraction (heart pumping measurement) (LVEF) above 50%. Hearing assessment and measured kidney clearance when indicated.
During	**Safety checks** NCCP	Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².
Response	**Cancer-specific checks** Individual specialist plan	There is no universal serum tumour-marker panel for this regimen. Histology, molecular testing and scans are chosen by the salivary gland multidisciplinary team.

Between treatments

- Symptoms can occur on any day; use your written contact plan.
- Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.

Bring your medicines list, calendar and verified contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fatigue and neurological symptoms

CAP

Balance activity and rest. Sudden confusion, seizure, severe headache or a change in vision needs urgent help.

2 Mouth, eating and swallowing

CAP

Report ulcers, pain, dry mouth, difficulty swallowing or weight loss early. Ask for oral care, dietetic and swallowing support.

3 Blood counts and infection

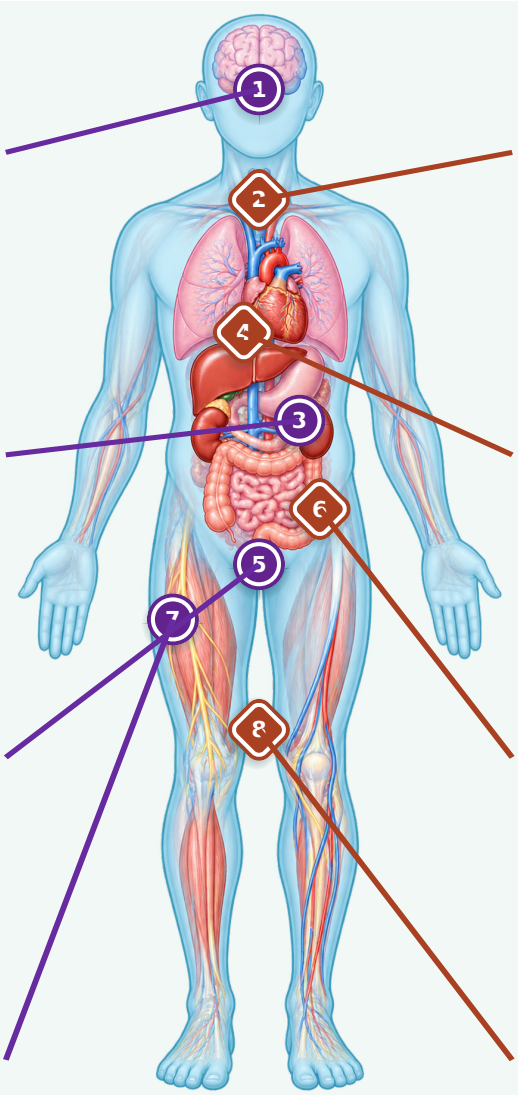
CAP

Fever, shivering, sudden illness, unusual bruising or bleeding needs urgent oncology advice.

4 Heart, hearing and nerves

Doxorubicin / cisplatin

Report breathlessness, chest symptoms, hearing changes or tingling promptly. Heart, hearing and nerve injury can persist.



5 Sickness and bowels

CAP

Use prescribed support. Report persistent diarrhoea or vomiting, inability to drink, blood or severe abdominal pain.

6 Kidneys and fluid balance

CAP

Report reduced urine, swelling or poor fluid intake; keep blood and urine tests.

7 Hair and infusion site

Doxorubicin / cyclophosphamide

Hair loss may occur. Tell the nurse immediately about burning, pain, swelling or leakage at the infusion site.

8 Fertility and pregnancy

CAP

Discuss fertility preservation, contraception, pregnancy and breastfeeding before starting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

CAP

Day 1 every 21 days; up to 6 cycles

Phase: _____ Planned cycles: 6

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	C5	C6
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.